

1. Administrative & Study Identification

Full Study Title: AN INTERVENTIONAL PHASE 1B/2 STUDY TO EVALUATE THE SAFETY AND EFFICACY OF PF-08634404 MONOTHERAPY AND IN COMBINATION WITH OTHER ANTICANCER AGENTS IN ADULT PARTICIPANTS WITH LOCALLY ADVANCED OR METASTATIC RENAL CELL CARCINOMA **Short Title/Acronym:** Symbiotic-GU-08 / A Study to Learn About the Medicine Called PF-08634404 Dosed Alone and in Combination With Other Anticancer Therapies in Adults With Locally Advanced or Metastatic Renal Cell Cancer **Protocol Number:** C6461008 **NCT Number:** NCT07227415 **Sponsor Name:** Pfizer **Investigational Product:** PF-08634404 (SSGJ-707, PD-1 x VEGF Bispecific Antibody) **Phase of Development:** Phase 1/Phase 2 (Phase 1b/2) **Medical Monitor:** Pfizer CT.gov Call Center / Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and efficacy of PF-08634404 monotherapy and in combination with other anticancer agents by determining the confirmed Objective Response Rate (ORR). **Secondary Objectives:** To evaluate supporting efficacy endpoints including Duration of Response (DOR), Progression-Free Survival (PFS), and Overall Survival (OS).

2.2 Background & Rationale To address the medical necessity for new therapeutic options in previously untreated, locally advanced or metastatic Renal Cell Carcinoma (RCC). PF-08634404 is an investigational tetravalent bispecific antibody targeting PD-1 and VEGF pathways. The study aims to evaluate if this dual-targeting mechanism, alone or in combination with other anticancer medicines, produces a favorable safety profile and robust tumor response in this patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm / Multiple-arm (investigational monotherapy and combination arms) **Blinding:** Open-label **Randomization:** Assigned to intervention arms

3.2 Planned Enrollment & Duration Target \$N\$: 224 Number of Sites: Multiple global clinical study sites **Estimated Study Start:** 10-Feb-2026 **Estimated Primary Completion:** 02-Nov-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. 18 years of age or older at screening. 2. Locally advanced (not amenable to curative surgery or radiation therapy) or metastatic RCC with diagnosis confirmed by histology/cytology. 3. At least one measurable (as defined by the investigator) and untreated lesion. 4. Adequate hematologic, hepatic, cardiac, and renal function. 5. No prior systemic therapy for RCC (immunotherapy after surgery is allowed if received >12 months prior). 6. Eastern Cooperative Oncology Group (ECOG) performance status 0 or 1.

4.2 Exclusion Criteria 1. Known active brain lesions including leptomeningeal metastasis, brainstem, meningeal or spinal cord metastases or compression. 2. Clinically significant risk of hemorrhage or fistula. 3. History of another malignancy within 3 years; History of allogeneic organ transplantation and allogeneic hematopoietic stem cell transplantation. 4. Active autoimmune diseases requiring systemic treatment within the past 2 years. 5. Uncontrolled cardiac and other comorbidities within 6 months prior to the first dose; Major surgery or severe trauma within 4 weeks before the first dose.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: PF-08634404 dosed according to Phase 1b escalation/Phase 2 expansion protocols, alongside Combination Drug 1 or 2 as applicable. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Until disease progression, unacceptable toxicity, withdrawal of consent, or study completion.

5.2 Concomitant & Prohibited Therapy Permitted: Pre-specified combination anticancer therapies (Drug Combination 1 and 2). **Prohibited:** Concurrent administration of other unapproved investigational therapies or systemic RCC therapies not specified in the protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Histology Confirmation, Baseline Tumor Imaging
Baseline	Day 0	Treatment Assignment, First IV Infusion, Vital Signs, Safety Labs	Interim Per Cycle Repeated IV Infusions, Safety Labs, Efficacy Assessments (RECIST v1.1 Scans)
End of Study	Follow-up	Final Vital Signs, Exit Interview, Monitoring for PFS and Overall Survival	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Confirmed objective response rate (ORR) defined as the proportion of participants achieving a Complete Response (CR) or Partial Response (PR). **Measurement Method:** Assessed by the investigator using Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1.

7.2 Secondary & Exploratory Endpoints

- 1. Duration of Response (DOR):** Time from the first documentation of objective response to the date of first documented disease progression or death.
- 2. Progression-Free Survival (PFS):** Time from randomization to the date of first documentation of objective progressive disease or death.
- 3. Overall Survival (OS):** Time from randomization to the date of death due to any cause.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent adverse events (TEAEs) at clinical study sites via patient interviews, physical exams, and lab assessments. **Serious Adverse Events (SAEs):** Standard oncology trial 24-hour reporting mechanisms. **Data Monitoring Committee (DMC):** Evaluated internally by Pfizer R&D.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (ORR, PFS, OS); Safety Set for adverse event profile characterization. **Sample Size Justification:** Planned to adequately evaluate safety dose-escalation and establish statistical confidence in the ORR for the expansion cohorts. **Handling of Missing Data:** Standard oncology data handling; patients dropping out prior to response evaluation are typically considered non-responders for ORR.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine remote and onsite monitoring visits by the sponsor (Pfizer) at all participating sites. **Audit Trail:** Secure Electronic Case Report Form (eCRF) locking and comprehensive data querying system.

11. Study Identifiers & Governance

Primary ID: C6461008 **Registry Number:** NCT07227415 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study to Learn About the Medicine Called PF-08634404 Dosed Alone and in Combination With Other Anticancer Therapies in Adults With Locally Advanced or Metastatic Renal Cell Cancer **Official Regulatory Title:** AN INTERVENTIONAL PHASE 1B/2 STUDY TO EVALUATE THE SAFETY AND EFFICACY OF PF-08634404 MONOTHERAPY AND IN COMBINATION WITH OTHER ANTICANCER AGENTS IN ADULT PARTICIPANTS WITH LOCALLY ADVANCED OR METASTATIC RENAL CELL CARCINOMA

12. Clinical Framework (Oncology Specific)

Primary Condition: Locally Advanced or Metastatic Renal Cell Carcinoma (RCC) **Diagnosis Threshold:** Histologically or cytologically confirmed measurable disease **Medical Methodology:** Bispecific Antibody Therapy (PD-1 x VEGF inhibition) **Optimization Target:** Tumor regression and durable clinical response (ORR, PFS)

13. Study Procedures & Methodology

Management Pathway: Clinical oncology pathway for intravenous bispecific antibodies. **Education Protocol (HCP):** Investigator training on administration protocols and identification of immune-related and VEGF-related AEs. **Education Protocol (Patient):** Informed consent process detailing the mechanisms of IV administration and potential toxicities. **Quality Audit Mechanism:** Independent radiological review and centralized response tracking (RECIST v1.1).

14. Observation & Follow-Up Schedule

Total Duration: Through primary completion (Est. Nov 2027) to study completion (Est. Nov 2028). **Onsite Visit Cadence:** Corresponds with intravenous infusion schedules and serial imaging evaluation intervals (e.g., every 6-8 weeks). **Remote Monitoring Frequency:** Between treatment cycles for survival follow-up. **Checklist Review Interval:** Prior to every infusion cycle.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	____	[DD-MMM-YYYY]				